SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Vinorelbine IV · advanced breast cancer

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Advanced breast cancer · NCCP 00232 · version 5 · 21-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
Aim to slow or shrink advanced breast cancer and help symptoms while balancing side effects. Duration depends on benefit, tolerability and the agreed plan; a fixed number of cycles or a guaranteed survival improvement is not promised.

**Schedule**
30 mg/m² onDays 1 and 8 every 21 days. Consultant may consider 25 mg/m² for poor marrow reserve; the dose is individualised after toxicity. Repeated 21-day cycles until progression/unacceptable toxicity; team review determines continuation.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

### Vinorelbine
VINCA ALKALOID CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Constipation or bowel obstruction
- ☐ Nerve effects and uncommon lung/heart toxicity
- ☐ Infusion-site injury

Purpose and alternatives for the person's advanced breast cancer, including other systemic treatment or supportive care according to goals. Possible benefit is uncertain; response is reviewed and treatment can be paused, changed or stopped. Low blood counts, infection/sepsis, fatigue, nausea/vomiting, bowel problems, neuropathy, hair change and uncommon serious lung/heart/allergic toxicity. Fertility and pregnancy planning before treatment; discuss sperm/egg preservation where relevant. Do not breastfeed. HPRA supplements: contraception during treatment and for 7 months afterwards for women,4 months for men; confirm the exact product and individual advice.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00232 v5.

SACTCheck v0.84.20
Developed by Dr Paul O'Brien